

1. Administrative & Study Identification

Full Study Title: A Multicenter Non-Interventional Study Evaluating Bleeds and Health-Related Quality Of Life in Patients With Type 3 Von Willebrand Disease on Prophylactic Standard-of-Care Treatment **Short Title/Acronym:** WILL-EMI NIS **Protocol Number:** WP45335
NCT Number: NCT06883240 **Sponsor Name:** Roche (Hoffmann-La Roche)
Trial Status: RECRUITING **Investigational Product / Associated Therapies:** Standard-of-Care (SOC) Prophylactic Therapy (e.g., Von Willebrand Factor Concentrates, Factor VIII Concentrates, Recombinant Activated Factor VII, thrombin). Trade names associated with interventions may include Evicel, Artiss, Recothrom, TachoSil, Evithrom, Thrombin-JMI, Tisseel, Thrombogen, Vistaseal, EVARREST, and Raplixa. **Phase of Development:** Not Applicable (Observational / Non-interventional study) **Medical Monitor:** Roche Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To collect information on the real-world effectiveness and safety of treatments received in routine clinical care, and to measure the health-related quality of life (HRQoL) of participants with Type 3 von Willebrand disease (VWD) receiving prophylactic therapy. **Secondary Objectives:** To document the Annualized Bleed Rate (ABR) for all bleeds, treated spontaneous bleeds, and treated joint bleeds, and record any adverse events occurring during the observation period.

2.2 Background & Rationale To address the need for comprehensive real-world data regarding the current clinical management of Type 3 VWD. This study is being conducted to establish a baseline understanding of bleeding frequency and quality of life for patients utilizing existing standard-of-care (SOC) prophylaxis, which serves as an important intra-patient comparator for evaluating future targeted therapies (such as emicizumab prophylaxis in subsequent interventional studies).

3. Study Design & Methodology

3.1 Design Framework Study Type: Observational (Non-interventional cohort study) **Trial Status:** RECRUITING **Control Type:** Single-arm (Observational) **Blinding:** Open-label **Randomization:** N/A

3.2 Planned Enrollment & Duration Target \$N\$: 40 participants
Number of Sites: Multicenter (e.g., UC Davis, University of Florida, University of Minnesota, Washington University)
Estimated Study Start: 29-Apr-2025 **Estimated Primary Completion:** 01-Nov-2026

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Confirmed diagnosis of Type 3 von Willebrand disease (VWD), based on medical records. 2. Adequate hematologic, hepatic, and renal function. 3. Documented and confirmed previous use of SOC prophylactic therapy for VWD (1-3 times weekly, as per prescribed dose) and anticipation to remain on the same regimen during the study. 4. For participants of childbearing potential: agreement to remain abstinent or adhere to the contraception requirements.

4.2 Exclusion Criteria 1. Inherited or acquired bleeding disorder other than Congenital Type 3 VWD. 2. History of gastrointestinal bleeding within 18 months prior to enrollment, or any previous diagnosis of angiodysplasia. 3. History of intracranial hemorrhage. 4. Previous or current treatment for thromboembolic disease or signs of thromboembolic disease. 5. Other conditions (e.g., certain autoimmune diseases) that may increase the risk of bleeding or thrombosis. 6. History of clinically significant hypersensitivity associated with monoclonal antibody therapies or components of emicizumab injection. 7. Use of systemic immunomodulators (e.g., interferon) at enrollment or planned use during the study, with the exception of anti-retroviral therapy.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Therapy: Standard of Care (SOC) prophylaxis therapy. Associated therapeutic drug classes may include thrombin (ATC Codes: B02BD30, B02BC06). **Dosage/Frequency:** Administered 1 to 3 times weekly as per prescribed dose. Dosing and treatment duration are at the discretion of the treating physician in accordance with local labeling or local treatment guidelines. **Route of Administration:** Intravenous (IV) - standard for VWF/FVIII factor concentrates. Local topical variants may be used depending on precise hemostatic product indication. **Duration of Treatment:** Observation period of at least 24 weeks.

5.2 Concomitant & Prohibited Therapy Permitted: Routine concomitant therapies deemed necessary by the treating physician for comorbid conditions. **Prohibited:** Investigational medications that may interfere with hemostasis; concurrent treatments for thromboembolic disease.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -14 to 0	Informed Consent, Medical History Review, Eligibility Confirmation
Baseline	Day 0	Enrollment, Initiation of HRQoL Questionnaires, Bleeding Log Initialization
Observation	Weeks 1 to 24	Routine clinical care visits, Ongoing documentation of bleeding events and SOC dosing
End of Study	Week 24 ($\pm$ 7)	Final Bleeding Log Review, Final HRQoL Assessment, Exit Interview

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Annualized Bleed Rate (ABR) for Treated Bleeds. Effectiveness of routine clinical care is evaluated via this metric, alongside Health-Related Quality of Life (HRQoL) scores over an observation period of at least 24 weeks. **Measurement Method:** Patient-reported bleeding logs, clinical assessment records, and validated HRQoL patient questionnaires.

7.2 Secondary & Exploratory Endpoints

- * Annualized Bleed Rate (ABR) for All Bleeds.
- * Annualized Bleed Rate (ABR) for Treated Spontaneous Bleeds.
- * Annualized Bleed Rate (ABR) for Treated Joint Bleeds.
- * Incidence and severity of Adverse Events (AEs) and Serious Adverse Events (SAEs) under real-world SOC.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Method of collection relies on routine clinical care tracking, patient diaries/interviews during standard follow-ups. **Serious Adverse Events (SAEs):** Reported by the site investigator within 24 hours of clinical awareness. **Data Monitoring Committee (DMC):** Internal safety monitoring committee overseen by the Sponsor (Roche) for ongoing pharmacovigilance.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Observational Cohort (All enrolled participants who received at least one dose of SOC prophylaxis during the 24-week period). **Sample Size Justification:** A target of 40 participants provides adequate descriptive statistical power to capture baseline bleeding rates and HRQoL metrics in this rare disease population. **Handling of Missing Data:** Multiple Imputation (MI) or Last Observation Carried Forward (LOCF) for incomplete HRQoL assessments; descriptive analysis for ABR calculations.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP), the Declaration of Helsinki, and applicable local regulations for non-interventional studies.

Monitoring Plan: Periodic remote and onsite data verification of electronic case report forms (eCRFs) against source medical records. **Audit Trail:** Standardized electronic data capture (EDC) system with full audit trails for data entry, querying, and final database lock.

11. Study Identifiers & Governance

Primary ID: WP45335 **Posting ID:** 290416627063928814 **Registry Number:** NCT06883240 **Sponsor/Lead Organization:** Roche **Study Title:** An Observational Study of Participants With Type 3 Von Willebrand Disease on Prophylactic Standard-of-Care Treatment (WILL-EMI NIS) **Official Regulatory Title:** A Multicenter Non-Interventional Study Evaluating Bleeds and Health-Related Quality Of Life in Patients With Type 3 Von Willebrand Disease on Prophylactic Standard-of-Care Treatment

12. Clinical Framework (Type 3 VWD Specific)

Primary Condition: Type 3 von Willebrand disease (VWD) **Diagnosis Threshold:** Confirmed diagnosis of Congenital Type 3 VWD via medical records and hematologic functioning **Medical Methodology:** Prophylactic Standard-of-Care (SOC) Therapy **Optimization Target:** Prevention of bleeding episodes and improvement of Health-Related Quality of Life (HRQoL)

13. Study Procedures & Methodology

Management Pathway: Routine clinical care pathways for bleeding disorders **Education Protocol (HCP):** Dosing in accordance with standard local labeling and clinical treatment guidelines **Education Protocol (Patient):** Routine counseling regarding prophylactic regimen adherence and bleed reporting **Quality Audit Mechanism:** Tracking via patient-reported bleeding logs and routine HRQoL questionnaire completion

14. Observation & Follow-Up Schedule

Total Duration: At least 24 observation weeks **Onsite Visit Cadence:** Dictated by the patient's routine clinical care schedule (typically including Baseline and Week 24) **Remote Monitoring Frequency:** Ongoing via real-time patient-reported outcome (PRO)

log entries **Checklist Review Interval:** Routine intervals per local site practices and End of Observation period

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				